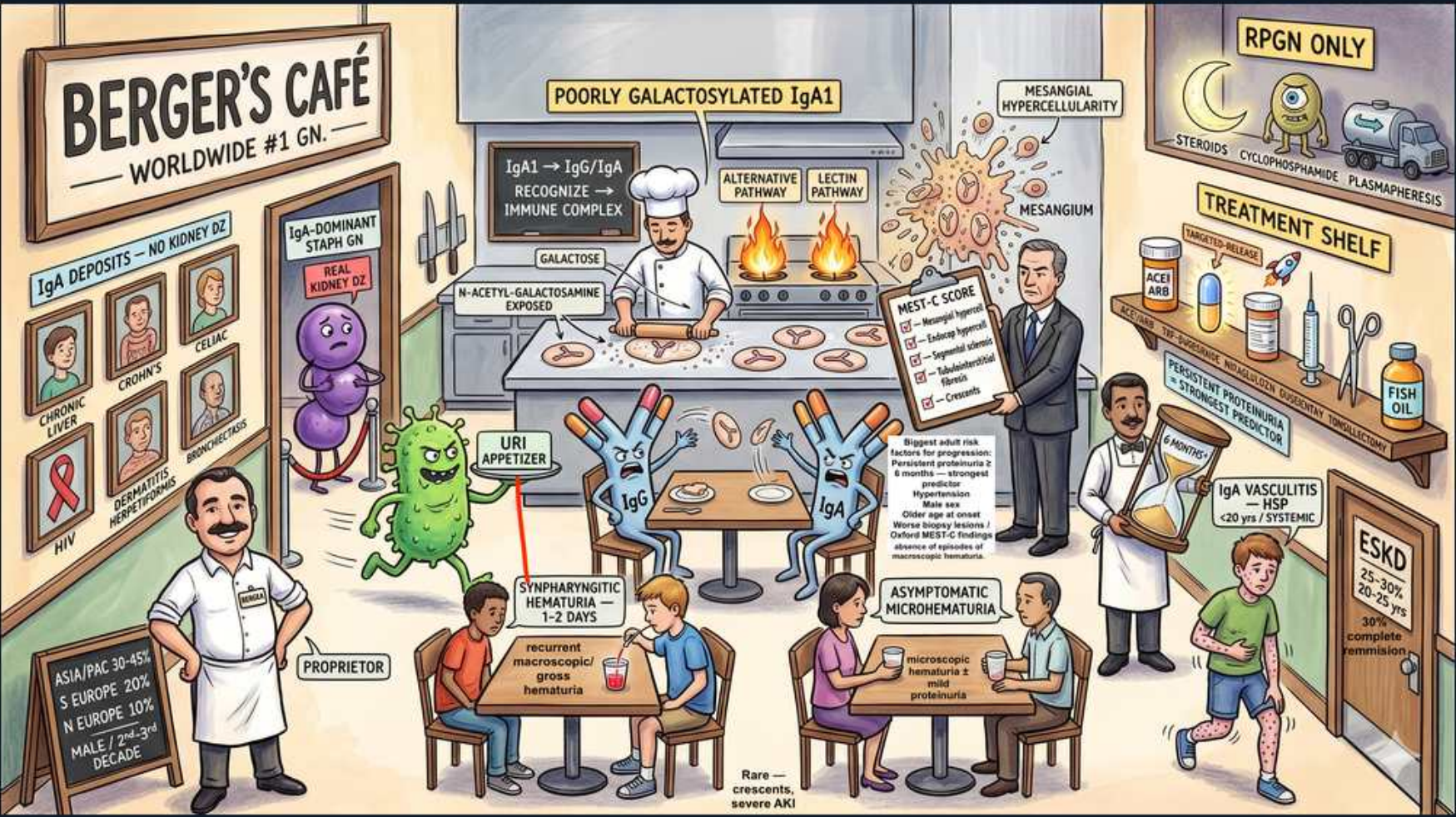


IgA Nephropathy (Berger's Disease)



1. Most common GN worldwide

WHAT YOU SEE

Berger's Café sign — 'worldwide #1 GN'

WHAT IT ENCODES

IgA nephropathy is the most common primary glomerulonephritis worldwide, especially in Asians and whites. Peaks in the 2nd–3rd decades, male predominance. Berger's disease.

BOARD TRAP

'Most common GN worldwide' = IgA; 'most common cause of nephrotic syndrome in adults' = FSGS/membranous — don't conflate the superlatives.

2. Poorly galactosylated IgA1

WHAT YOU SEE

Chef banner 'poorly galactosylated IgA1'

WHAT IT ENCODES

Pathogenesis: galactose-deficient IgA1 (Gd-IgA1) is recognized by IgG/IgA autoantibodies forming immune complexes that deposit in the mesangium, triggering mesangial proliferation and complement (alternative/lectin pathway) activation. Four-hit model.

BOARD TRAP

Serum IgA is elevated in only ~50% — a normal IgA level does NOT exclude IgA nephropathy; diagnosis is by biopsy.

3. Mesangial IgA deposits

WHAT YOU SEE

IgA deposits panel, upper left

WHAT IT ENCODES

Immunofluorescence shows DOMINANT/co-dominant mesangial IgA deposits (often with C3 and IgG). EM: mesangial electron-dense deposits. Light microscopy: mesangial hypercellularity ± matrix expansion.

BOARD TRAP

Mesangial IgA on IF is also seen in IgA vasculitis (HSP) and cirrhosis — the renal-limited biopsy looks identical; clinical context separates them.

4. Synpharyngitic hematuria

WHAT YOU SEE

Synpharyngitic hematuria — 1–2 days, lower center

WHAT IT ENCODES

Classic presentation: gross hematuria CONCURRENT with (within 1–2 days of) an upper respiratory or GI infection — 'synpharyngitic.' Reflects mucosal IgA production responding to the trigger.

BOARD TRAP

Post-strep GN hematuria is DELAYED 1–3 weeks after infection; IgA's hematuria is synpharyngitic (same time) — the latency gap is the discriminator.

5. Asymptomatic microhematuria

WHAT YOU SEE

Asymptomatic microhematuria diner, center

WHAT IT ENCODES

Many patients present incidentally with persistent microscopic hematuria ± mild proteinuria and preserved GFR. Dysmorphic RBCs and RBC casts indicate glomerular origin. Often a benign indolent course in this subgroup.

BOARD TRAP

Isolated microhematuria with normal function is often benign IgA — but proteinuria >1 g/day, hypertension, and reduced GFR predict progression.

6. URI trigger

WHAT YOU SEE

URI 'appetizer' near the IgA hands

WHAT IT ENCODES

Mucosal infections (URI, gastroenteritis) drive mucosal IgA1 production and trigger gross hematuria flares. This mucosal link underlies the synpharyngitic timing.

BOARD TRAP

Tonsillectomy is NOT standard of care in Western guidelines despite Japanese data — don't pick it as a board treatment answer.

7. MEST-C score

WHAT YOU SEE

MEST-C score board, center

WHAT IT ENCODES

Oxford MEST-C histologic classification scores Mesangial hypercellularity, Endocapillary hypercellularity, Segmental sclerosis, Tubular atrophy/interstitial fibrosis, and Crescents — each carries prognostic weight and guides therapy.

BOARD TRAP

Crescents (C) and endocapillary proliferation flag aggressive disease needing immunosuppression — bland mesangial-only disease usually does not.

8. Treatment shelf (RAAS first)

WHAT YOU SEE

Treatment shelf — supportive meds, fish oil

WHAT IT ENCODES

First-line is optimized supportive care: RAAS blockade (ACEi/ARB) for proteinuria/BP, SGLT2 inhibitors, and BP control. Persistent proteinuria >0.75–1 g/day despite this may warrant targeted-release budesonide or systemic steroids; sparsentan is an option.

BOARD TRAP

Reflex high-dose steroids for all IgA is wrong — maximize RAAS/supportive therapy first; immunosuppression is for high-risk progressors.

9. IgA vasculitis (HSP) link

WHAT YOU SEE

IgA vasculitis — HSP, systemic panel, right

WHAT IT ENCODES

IgA vasculitis (Henoch-Schönlein purpura) is the systemic form: palpable purpura (lower limbs/buttocks), arthralgia, abdominal pain, and IgA nephropathy. Same mesangial IgA deposition; more common in children, usually self-limited.

BOARD TRAP

HSP = IgA nephropathy WITH systemic small-vessel vasculitis — purpura + abdominal pain + nephritis in a child is HSP, not isolated Berger's.

10. RPGN with crescents

WHAT YOU SEE

'RPGN only — steroids/cyclophosphamide/plasmapheresis' banner top right

WHAT IT ENCODES

A minority develop crescentic, rapidly progressive GN with rising creatinine. This subset is treated like a crescentic GN: high-dose steroids ± cyclophosphamide ± plasmapheresis.

BOARD TRAP

Aggressive immunosuppression is reserved for crescentic/RPGN IgA — applying it to bland microhematuria over-treats and harms.

11. ESKD risk over decades

WHAT YOU SEE

ESKD card, lower right

WHAT IT ENCODES

IgA nephropathy is chronic: roughly 20–40% progress to ESKD over 10–20 years. Predictors: proteinuria >1 g/day, hypertension, reduced eGFR at diagnosis, and adverse MEST-C features.

BOARD TRAP

IgA is not uniformly benign — the indolent label applies to isolated microhematuria, not to proteinuric/hypertensive disease which steadily progresses.

12. IgA-dominant infection GN mimic

WHAT YOU SEE

'IgA-dominant — no kidney disease?' staph note, left

WHAT IT ENCODES

IgA-DOMINANT infection-related GN (often Staph aureus, in diabetics/elderly) can mimic primary IgA nephropathy on IF. Look for active infection, low complement, and an acute deteriorating course rather than chronic indolent disease.

BOARD TRAP

Staph-associated IgA-dominant GN is an INFECTION mimicking Berger's — treating with immunosuppression instead of antibiotics worsens it.
